KET QUA SINH HOA MAU

Patient:		Species:	Canine	Patient ID:	
Client:	THANH	Gender:	Female	Sample No.:	84
Doctor:		Age:	Adult 4Y	Time of analysis:	2026/03/06 12:44

	Item		Current result		Ref. Ranges
Protein	**TP**		**59.8**	g/L	53.1-79.2
Protein	**ALB**		**29.6**	g/L	23.4-40.0
Protein	**GLOB**		**30.2**	g/L	25.4-52.0
Protein	**A/G**		**1.0**		
Liver and gallbladder	**ALT**	↑	**313.8**	U/L	10.1-100.3
Liver and gallbladder	**AST**	↑	**72.9**	U/L	0.0-51.7
Liver and gallbladder	**AST/ALT**		**0.23**		
Liver and gallbladder	**ALP**	↑	**1090.1**	U/L	15.5-212.0
Liver and gallbladder	**GGT**	↑	**57.0**	U/L	0.0-15.9
Liver and gallbladder	**TBIL**		**<1.70**	μmol/L	0.00-15.00
Liver and gallbladder	**TBA**		**11.6**	μmol/L	0.0-30.0
Pancreas	**AMY**		**486.1**	U/L	397.7-1285.1
Kidneys	**BUN**		**8.88**	mmol/L	2.50-9.77
Kidneys	**CREA**		**31.20**	μmol/L	20.00-123.70
Kidneys	**BUN/CREA**		**70.5**		
Cardiovasc./Muscle	**CK**		**145.6**	U/L	66.4-257.5
Cardiovasc./Muscle	**LDH**		**139.6**	U/L	0.0-143.6
Energy metabolism	**GLU**	↑	**7.71**	mmol/L	3.80-7.50
Energy metabolism	**TC**		**4.26**	mmol/L	2.67-8.38
Energy metabolism	**TG**	↑	**1.37**	mmol/L	0.10-1.30
Minerals	**Ca**		**2.42**	mmol/L	2.10-2.97
Minerals	**PHOS**		**1.95**	mmol/L	0.80-2.20
Minerals	**CaxP**		**4.70**	mmol/L^2	
Minerals	**Mg**		**0.87**	mmol/L	0.53-1.06
Electrolytes	**Na+**		**151.5**	mmol/L	138.0-160.0
Electrolytes	**K+**		**4.8**	mmol/L	3.5-5.9
Electrolytes	**Na/K**		**31.6**		
Electrolytes	**Cl-**	↑	**126.5**	mmol/L	102.7-125.0

Operator:

Comprehensive Diagnosis Panel **QC QC OK**

HEM(Hemolysis degree):	0	LIP(Lipemia degree):	0	ICT(Jaundice degree):	0

Report Explan.

ALT	↑	Increase is commonly associated with liver injury and muscle injury, etc.
AST	↑	Increase is commonly associated with liver injury and muscle injury, etc.

The results only applies to this test sample.

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KET QUA SINH HOA MAU

Patient:		Species:	Canine	Patient ID:	
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Doctor:		Age:	Adult 4Y	Time of analysis:	2026/03/06 12:44

Report Explan.

ALP	↑	Increase is commonly associated with fracture healing period, hepatobiliary diseases, hyperthyroidism, and osteosarcoma, etc.
GGT	↑	Elevated is commonly associated with bile duct injury or cholestasis, etc.
GLU	↑	Increase is commonly associated with diabetes and hypercorticalismus, etc. Reduction is commonly associated with insulin administration, malnutrition, and insulinoma, etc.
TG	↑	Increase is commonly associated with postprandial, obesity, diabetes and hypercorticalismus, etc.
Cl-	↑	Increase is commonly associated with salt intoxication, hypertonic NaCl solution rehydration, small intestinal diarrhea, etc. Reduction is commonly associated with vomiting, diuretic therapy, etc.

Note: Due to the complexity and individuality of disease diagnosis, the report interpretation is only for your reference. Please consult your doctors for clinical diagnosis results.

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